

2024, Mar. 2024 Release LOC:Acute Adult

Acetaminophen Overdose

Utilization Benchmarks: Multiple options available, see table below for options

Overview

Select Day

Initial review (1, 2, 3, 4)

Episode Day 1 (1, 3, 4)

Episode Day 2 (3, 4, 17)

Episode Day 3 (3, 4, 27)

Episode Day 4

Discharge Screens (28)

Utilization Benchmarks

Notes

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Overview

Instruction: This subset is for patients who have an accidental or intentional acetaminophen overdose. For patients who have consumed a less than toxic dose of acetaminophen as a suicidal gesture, see "Danger to self or others" criteria in the General Medical subset.

Introduction:

Acetaminophen (APAP) is the most commonly used over-the-counter analgesic and antipyretic worldwide and is present in many pharmaceutical products. Whether taken accidentally or intentionally, APAP overdose is the leading cause of acute liver failure in the United States. Hepatotoxicity resulting from an APAP overdose can occur acutely after a single ingestion of a supratherapeutic dose, or over time, following repeated supratherapeutic doses, where it may go undetected until symptoms develop.

Patients presenting with APAP overdose may not show signs of immediate toxicity and liver function test (LFT) abnormalities are usually not detectable sooner than 24 hours post ingestion. Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) levels in excess of 1000 U/L (16.7 μ kat/L) are indicative of severe hepatotoxicity. When the time of ingestion is known, the Rumack-Matthew nomogram is used to estimate the likelihood of hepatotoxicity in patients greater than 12 years of age. Treatment of APAP overdose with N-acetylcysteine (NAC) should be initiated within 8 hours of APAP ingestion. Patients with hepatic injury from APAP overdose may present with nausea, vomiting, confusion, right upper quadrant pain, scleral icterus, and elevated liver enzymes, which typically increase 24 to 48 hours post ingestion. Within 3 to 5 days, jaundice, coagulopathy, and encephalopathy may develop. Without treatment, hepatotoxicity may progress to multi-organ failure and death. Patients at higher risk for liver failure are those with chronic alcohol use, malnutrition, prolonged fasting, dehydration, are pregnant, or who may have a lower threshold for APAP toxicity. Most deaths from liver failure occur within the first week following an APAP overdose.

Evaluation and treatment:

The antidote for APAP toxicity is N-acetylcysteine (NAC) which is typically administered according the Rumack-Matthew nomogram. The nomogram is a useful tool for cases of acute ingestion but is less useful in cases of chronic or repeated ingestion. NAC is maximally hepatoprotective when given within 8 hours of an acute APAP overdose but is also beneficial in the treatment of patients with a late presentation and signs of hepatic injury. NAC can be given intravenously or orally. The selected route and dosage of treatment is given at the discretion of the provider.

A number of screening tools are available to define the risk of acute liver failure after an APAP overdose and identify patients that may subsequently require liver transplantation. The King's College Hospital (KCH) and Modified KCH are the most widely used tools (Darr and Sussman, Clin Liver Dis 2020, 24: 121-9; Fisher and Curry, Adv Pharmacol 2019, 85: 263-72; Chiew et al., Cochrane Database Syst Rev 2018, 2: CD003328; McPhail et al., Clin Gastroenterol Hepatol 2016; 14(4): 516-525 e515; e543-e545; McNeil Consumer Healthcare, Guidelines for the Management of Acetaminophen

Overdose. 2015; Bernal et al., Lancet 2002, 359: 558-63; O'Grady et al., Gastroenterology 1989, 97: 439-45).

InterQual® criteria are derived from the systematic, continuous review and critical appraisal of the most current evidence-based literature and include input from our independent panel of clinical experts. To generate the most appropriate recommendations, a comprehensive literature review of the clinical evidence was conducted. Sources searched included PubMed, Agency for Healthcare Research and Quality (AHRQ) Comparative Effectiveness Reviews, the National Institute for Health and Care Excellence (NICE), the Centers for Medicare and Medicaid Services (CMS), The Cochrane Library, and The Joint Commission. Other medical literature databases, medical content providers, data sources, regulatory body websites, and specialty society resources may also have been utilized. Relevant studies were assessed for risk of bias following principles described in the *Cochrane Handbook*. The resulting evidence was assessed for consistency, directness, precision, effect size, and publication bias. Observational trials were also evaluated for the presence of a dose-response gradient and the likely effect of plausible confounders. The content is based on a variety of references which are cited at specific criteria points throughout the subset.

(Excludes PO medications unless noted)

Select Day, One:● **Initial review, One:** ^(1, 2, 3, 4)

(From arrival in ED through decision to admit)

(Symptom or finding within 24h)

● **OBSERVATION, One:** ⁽⁵⁾– N-Acetylcysteine (NAC) (includes IV, PO or NG) ^(6, 7, 8)● **ACUTE, Both:** ⁽⁹⁾● **Finding, ≥ One:**– Abdominal pain, right upper quadrant ⁽¹⁰⁾– Aspartate aminotransferase (AST) > 50 U/L(0.84 µkat/L) ^(10, 11)– Alanine aminotransferase (ALT) > 50 U/L(0.84 µkat/L) ^(10, 11)– Vomiting, persistent and unresponsive to ≥ 2 doses antiemetic ⁽¹⁰⁾– N-Acetylcysteine (NAC) (includes IV, PO or NG) ^(7, 8)● **CRITICAL, Both:** ⁽¹²⁾● **Finding, ≥ One:**– Aspartate aminotransferase (AST) > 1000 U/L(16.7 µkat/L) ^(11, 13)– Alanine aminotransferase (ALT) > 1000 U/L(16.7 µkat/L) ^(11, 13)– Creatinine ≥ 3.3 mg/dL ⁽¹³⁾– INR > 5.0 ⁽¹³⁾– Hepatic encephalopathy Grade III or IV ^(13, 14, 15)– Mental status change ^(13, 16)– N-Acetylcysteine (NAC) (includes IV, PO or NG) ^(7, 8)● **Episode Day 1, One:** ^(1, 3, 4)

(Symptom or finding within 24h)

● **OBSERVATION, One:** ⁽⁵⁾– N-Acetylcysteine (NAC) (includes IV, PO or NG) ^(6, 7, 8)● **ACUTE, Both:** ⁽⁹⁾● **Finding, ≥ One:**– Abdominal pain, right upper quadrant ⁽¹⁰⁾– Aspartate aminotransferase (AST) > 50 U/L(0.84 µkat/L) ^(10, 11)– Alanine aminotransferase (ALT) > 50 U/L(0.84 µkat/L) ^(10, 11)– Vomiting, persistent and unresponsive to ≥ 2 doses antiemetic ⁽¹⁰⁾– N-Acetylcysteine (NAC) (includes IV, PO or NG) ^(7, 8)● **CRITICAL, Both:** ⁽¹²⁾● **Finding, ≥ One:**– Aspartate aminotransferase (AST) > 1000 U/L(16.7 µkat/L) ^(11, 13)– Alanine aminotransferase (ALT) > 1000 U/L(16.7 µkat/L) ^(11, 13)– Creatinine ≥ 3.3 mg/dL ⁽¹³⁾– INR > 5.0 ⁽¹³⁾– Hepatic encephalopathy Grade III or IV ^(13, 14, 15)– Mental status change ^(13, 16)– N-Acetylcysteine (NAC) (includes IV, PO or NG) ^(7, 8)● **Episode Day 2, One:** ^(3, 4, 17)● **OBSERVATION, One:** ⁽⁵⁾● **Responder**, discharge expected today if clinically stable last 12h, **All:** ⁽¹⁸⁾

– Acetaminophen (APAP) level < 10 µg/mL(66.1 µmol/L)

– N-Acetylcysteine (NAC) administration completed or no longer indicated ⁽¹⁹⁾– Liver function tests (LFT) within acceptable limits ⁽²⁰⁾

– Behavioral health risk assessment completed or not indicated

– Tolerating PO ⁽²¹⁾● **Partial responder**, not clinically stable for discharge and requires continued stay, ≥ **One:**– N-Acetylcysteine (NAC) (includes IV, PO or NG) ^(6, 7, 8)

- Behavioral health risk assessment $\leq 24\text{h}$

● **ACUTE, One:**

● **Early responder**, discharge expected today if clinically stable last 12h, **All:** ⁽¹⁸⁾

- Acetaminophen (APAP) level $< 10\text{ }\mu\text{g/mL}$ ($66.1\text{ }\mu\text{mol/L}$)
- N-Acetylcysteine (NAC) administration completed or no longer indicated ⁽¹⁹⁾
- Liver function tests (LFT) within acceptable limits ⁽²⁰⁾
- Pain controlled or manageable
- Tolerating PO ⁽²¹⁾

● **Partial responder**, not clinically stable for discharge and requires continued stay, $\geq$ **One:**

● Signs and symptoms, **Both:**

● Finding, $\geq$ **One:**

- Abdominal pain, right upper quadrant ⁽¹⁰⁾
- Aspartate aminotransferase (AST) $>$ upper limit of normal (ULN) ^(10, 11)
- Alanine aminotransferase (ALT) $>$ upper limit of normal (ULN) ^(10, 11)
- Vomiting, persistent ⁽¹⁰⁾
- N-Acetylcysteine (NAC) (includes IV, PO or NG) ^(7, 8)
- Post Critical care $\leq 24\text{h}$

● **INTERMEDIATE, $\geq$ One:** ⁽²²⁾

- NIPPV ⁽²³⁾
- Oxygen $\geq 40\%$ (0.40), $\leq 2\text{d}$ ⁽²⁴⁾

● **CRITICAL, $\geq$ One:**

● Acetaminophen (APAP) overdose, **Both:** ⁽²⁵⁾

● Finding, $\geq$ **One:**

- Aspartate aminotransferase (AST) $> 1000\text{ U/L}$ ($16.7\text{ }\mu\text{kat/L}$) ^(11, 13)
- Alanine aminotransferase (ALT) $> 1000\text{ U/L}$ ($16.7\text{ }\mu\text{kat/L}$) ^(11, 13)
- Creatinine $\geq 3.3\text{ mg/dL}$ ⁽¹³⁾
- INR > 5.0 ⁽¹³⁾
- Hepatic encephalopathy Grade III or IV ^(13, 14, 15)
- Mental status change ^(13, 16)
- N-Acetylcysteine (NAC) (includes IV, PO or NG) ^(7, 8)
- Mechanical ventilation
- NIPPV and, $\geq$ **One:** ⁽²³⁾
 - $\text{FiO}_2 > 50\%$ (0.50)
 - Respiratory intervention every 1-2h ⁽²⁶⁾

● **Episode Day 3, One:** ^(3, 4, 27)

● **OBSERVATION, One:** ⁽⁵⁾

● **Responder**, discharge expected today if clinically stable last 12h, **All:** ⁽¹⁸⁾

- Acetaminophen (APAP) level $< 10\text{ }\mu\text{g/mL}$ ($66.1\text{ }\mu\text{mol/L}$)
- N-Acetylcysteine (NAC) administration completed or no longer indicated ⁽¹⁹⁾
- Liver function tests (LFT) within acceptable limits ⁽²⁰⁾
- Behavioral health risk assessment completed or not indicated
- Tolerating PO ⁽²¹⁾

● **Non-responder**, not clinically stable for discharge and exceeds episode days at the Observation level of care for this condition, **One:**

- For acetaminophen overdose use Episode Day 3 at a higher level of care
- For a condition other than acetaminophen overdose use appropriate subset (Episode Day 1) based on clinical findings
- Refer for secondary review

● **ACUTE, One:**

● **Responder**, discharge expected today if clinically stable last 12h, **All:** ⁽¹⁸⁾

- Acetaminophen (APAP) level $< 10\text{ }\mu\text{g/mL}$ ($66.1\text{ }\mu\text{mol/L}$)
- N-Acetylcysteine (NAC) administration completed or no longer indicated ⁽¹⁹⁾
- Liver function tests (LFT) within acceptable limits ⁽²⁰⁾

- Pain controlled or manageable
- Tolerating PO ⁽²¹⁾
- **Partial responder**, not clinically stable for discharge and requires continued stay, ≥ **One**:
 - Signs and symptoms, **Both**:
 - Finding, ≥ **One**:
 - Abdominal pain, right upper quadrant ⁽¹⁰⁾
 - Aspartate aminotransferase (AST) > upper limit of normal (ULN) ^(10, 11)
 - Alanine aminotransferase (ALT) > upper limit of normal (ULN) ^(10, 11)
 - Vomiting, persistent ⁽¹⁰⁾
 - N-Acetylcysteine (NAC) (includes IV, PO or NG) ^(7, 8)
 - Post Critical care ≤ 24h
- **INTERMEDIATE**, ≥ **One**:
 - NIPPV ⁽²³⁾
 - Oxygen ≥ 40%(0.40), ≤ 2d ⁽²⁴⁾
- **CRITICAL**, ≥ **One**:
 - Acetaminophen (APAP) overdose, **Both**: ⁽²⁵⁾
 - Finding, ≥ **One**:
 - Aspartate aminotransferase (AST) > 1000 U/L(16.7 µkat/L) ^(11, 13)
 - Alanine aminotransferase (ALT) > 1000 U/L(16.7 µkat/L) ^(11, 13)
 - Creatinine ≥ 3.3 mg/dL ⁽¹³⁾
 - INR > 5.0 ⁽¹³⁾
 - Hepatic encephalopathy Grade III or IV ^(13, 14, 15)
 - Mental status change ^(13, 16)
 - N-Acetylcysteine (NAC) (includes IV, PO or NG) ^(7, 8)
 - Mechanical ventilation
 - NIPPV and, ≥ **One**: ⁽²³⁾
 - FiO₂ > 50%(0.50)
 - Respiratory intervention every 1-2h ⁽²⁶⁾
- **Episode Day 4, One**:
 - **ACUTE, One**:
 - **Responder**, discharge expected today if clinically stable last 12h, **All**: ⁽¹⁸⁾
 - Acetaminophen (APAP) level < 10 µg/mL(66.1 µmol/L)
 - N-Acetylcysteine (NAC) administration completed or no longer indicated ⁽¹⁹⁾
 - Liver function tests (LFT) within acceptable limits ⁽²⁰⁾
 - Pain controlled or manageable
 - Tolerating PO ⁽²¹⁾
 - **Non-responder**, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, **One**:
 - Use Extended Stay subset
 - Refer for secondary review
 - **INTERMEDIATE, One**:
 - **Non-responder**, not clinically stable for discharge and exceeds episode days at the Intermediate level of care for this condition, **One**:
 - Use Extended Stay subset
 - Refer for secondary review
 - **CRITICAL, One**:
 - **Non-responder**, not clinically stable for discharge and exceeds episode days at the Critical level of care for this condition, **One**:
 - Use Extended Stay subset
 - Refer for secondary review

Discharge, Level of Care, One: ⁽²⁸⁾

- **HOME, Both**:

- Level of care appropriateness, **Both:**
 - Home environment safe and accessible ⁽²⁹⁾
 - Patient or caregiver demonstrates ability to manage care
- Acetaminophen (APAP) overdose discharge planning, **All:**
 - Provide comprehensive written discharge and teaching instructions ⁽³⁰⁾
 - Education on the importance of, **All:**
 - The potential risks associated with the inappropriate use of acetaminophen (APAP)
 - The increased risk for hepatotoxicity when using acetaminophen (APAP) and non-steroidal anti-inflammatory drugs (NSAIDs) or chronic alcohol use
 - Checking over the counter medication labels for acetaminophen (APAP) to avoid accidental overdose
 - Medication reconciliation ⁽³¹⁾
 - Follow-up care planned ⁽³²⁾
 - Identify and address transportation needs
- **HOME CARE, Both:**
 - Level of care appropriateness, **All:**
 - Home environment safe and accessible ⁽²⁹⁾
 - Patient or caregiver willing and able to learn care
 - Outpatient management contraindicated or unavailable ^(33, 34)
 - Skilled services, ≥ **One:** ⁽³⁵⁾
 - Skilled nursing ⁽³⁶⁾
 - Skilled therapy, PT, OT, or ST
 - Behavioral health ⁽³⁷⁾
 - Acetaminophen (APAP) overdose discharge planning, **All:**
 - Provide comprehensive written discharge and teaching instructions ⁽³⁰⁾
 - Education on the importance of, **All:**
 - The potential risks associated with the inappropriate use of acetaminophen (APAP)
 - The increased risk for hepatotoxicity when using acetaminophen (APAP) and non-steroidal anti-inflammatory drugs (NSAIDs) or chronic alcohol use
 - Checking over the counter medication labels for acetaminophen (APAP) to avoid accidental overdose
 - Medication reconciliation ⁽³¹⁾
 - Follow-up care planned ⁽³²⁾
 - Identify and address transportation needs
- **BEHAVIORAL HEALTH, All:**
 - Level of care appropriateness, **One:**
 - Support systems available ⁽³⁸⁾
 - Skilled treatment, ≥ **One:**
 - Clinical assessment ⁽³⁹⁾
 - Individual, group, or family counseling
 - Psychiatric, substance, or medication evaluation ⁽⁴⁰⁾
 - Acetaminophen (APAP) overdose discharge planning, **All:**
 - Provide comprehensive written discharge and teaching instructions ⁽³⁰⁾
 - Education on the importance of, **All:**
 - The potential risks associated with the inappropriate use of acetaminophen (APAP)
 - The increased risk for hepatotoxicity when using acetaminophen (APAP) and non-steroidal anti-inflammatory drugs (NSAIDs) or chronic alcohol use
 - Checking over the counter medication labels for acetaminophen (APAP) to avoid accidental overdose
 - Medication reconciliation ⁽³¹⁾
 - Follow-up care planned ⁽³²⁾
 - Identify and address transportation needs
- **SKILLED MEDICAL OR THERAPY, Both:**
 - Level of care appropriateness, **All:**
 - Medical practitioner, NP, PA assessment or oversight ≥ 1x/wk
 - Treatment precluded in a lower level

- **Services, ≥ One:**
 - Medical and skilled nursing interventions or assessment 1-2x/24h ⁽³⁶⁾
- **Therapy, All:**
 - Goal directed therapy with rehabilitation potential ^(41, 42)
 - Functional impairments requiring at least supervision ⁽⁴³⁾
 - Able to tolerate 1h/d of skilled therapy ≥ 5d/wk
- **Complete prior to facility transfer, All:**
 - Medication reconciliation ⁽³¹⁾
 - Obtain and complete forms for facility
 - Obtain discharge summary and transmit to facility and medical practitioner
 - Arrange transportation
- **SUBACUTE MEDICAL OR THERAPY, Both:**
 - **Level of care appropriateness, All:**
 - Medical practitioner, NP, PA assessment or oversight ≥ 2x/wk
 - Treatment precluded in a lower level
 - **Services, ≥ One:**
 - Medical and skilled nursing interventions or assessment 4h/24h ⁽³⁶⁾
 - **Therapy, All:**
 - Goal directed therapy with rehabilitation potential ^(41, 42)
 - ≥ 2 functional impairments requiring at least minimum assistance ⁽⁴³⁾
 - Able to tolerate 2-3h/d of skilled therapy ≥ 5d/wk and ≥ 1 therapy discipline
 - **Complete prior to facility transfer, All:**
 - Medication reconciliation ⁽³¹⁾
 - Obtain and complete forms for facility
 - Obtain discharge summary and transmit to facility and medical practitioner
 - Arrange transportation
- **SUBACUTE REHAB, Both:**
 - **Level of care appropriateness, All:**
 - Medical practitioner, NP, PA assessment or oversight ≥ 3x/wk
 - Treatment precluded in a lower level
 - **Services, All:**
 - Goal directed therapy with rehabilitation potential ^(41, 42)
 - ≥ 2 functional impairments requiring at least minimum assistance ⁽⁴³⁾
 - Able to tolerate 2-3h/d of skilled therapy ≥ 5d/wk and ≥ 2 therapy disciplines
 - **Complete prior to facility transfer, All:**
 - Medication reconciliation ⁽³¹⁾
 - Obtain and complete forms for facility
 - Obtain discharge summary and transmit to facility and medical practitioner
 - Arrange transportation
- **ACUTE REHAB, Both:**
 - **Level of care appropriateness, Both:**
 - Medical practitioner, NP, PA assessment or oversight ≥ 3x/wk
 - **Services, All:**
 - Goal directed therapy with rehabilitation potential ^(41, 42)
 - ≥ 2 functional impairments requiring at least minimum assistance ⁽⁴³⁾
 - Able to tolerate ≥ 15h/wk of skilled therapy and ≥ 2 therapy disciplines ⁽⁴⁴⁾
 - **Complete prior to facility transfer, All:**
 - Medication reconciliation ⁽³¹⁾
 - Obtain and complete forms for facility
 - Obtain discharge summary and transmit to facility and medical practitioner
 - Arrange transportation
- **LONG TERM ACUTE CARE, Both:**
 - **Level of care appropriateness, Both:**

- Treatment precluded in a lower level
- In lieu of acute or continued hospitalization or failed lower level of care, ≥ **One**:
 - Primary condition or illness and treatment of active comorbid condition(s) ⁽⁴⁵⁾
 - Ventilator dependent ≥ 6h/d and weaning planned ^(46, 47)
 - Acute and comorbid conditions requiring prolonged hospitalization ⁽⁴⁸⁾
- Complete prior to facility transfer, **All**:
 - Medication reconciliation ⁽³¹⁾
 - Obtain and complete forms for facility
 - Obtain discharge summary and transmit to facility and medical practitioner
 - Arrange transportation

Utilization Benchmarks

Condition or Procedure	% Pd Obs	LOS (days)	Type
917 POISONING AND TOXIC EFFECTS OF DRUGS WITH MCC	n/a	3.9	CMS GMLOS
918 POISONING AND TOXIC EFFECTS OF DRUGS WITHOUT MCC	n/a	2.5	CMS GMLOS
922 OTHER INJURY, POISONING AND TOXIC EFFECT DIAGNOSES WITH MCC	n/a	4.5	CMS GMLOS
923 OTHER INJURY, POISONING AND TOXIC EFFECT DIAGNOSES WITHOUT MCC	n/a	2.9	CMS GMLOS
ACETAMINOPHEN OVERDOSE	24%	3.1	InterQual

Percent Paid as Observation indicates the final payment status as a percentage for patients hospitalized with the condition. It may not correspond to the status assigned at admission. Note that patients initially placed in Observation status who are converted to inpatient status (common) are included in the inpatient category, and that patients initially admitted to inpatient status who are converted to observation status (uncommon) are included in the Observation category.

Notes:**1:**

The CMS Two-Midnight Rule generally requires that the medical record support a patient's need for hospital-level services **AND** that the patient has a hospital stay of at least two midnights. Short-stay exceptions to this rule are defined below. Application of InterQual® criteria can help an organization comply with the medical necessity requirements of the two-midnight rule.

For those who are impacted by the CMS Two-Midnight Rule, follow the steps below:

1. **Determine the need for hospital-based services by applying criteria.** If a patient meets criteria at any level of care, they have met the medical necessity requirement of the rule.
2. **Determine whether the patient is an inpatient or assign observation status** depending on which level of care is met and the expected length of stay. Based on the presentation of the patient, evidence-based criteria such as InterQual® can support the provider's expectation for length of stay.
 - **When care is expected to span two midnights** and the patient has met criteria at the Acute, Intermediate or Critical level of care, an inpatient status may be appropriate.

NOTE: In the event of an "Unforeseen Circumstance" per CMS resulting in a shorter stay than initially expected, a status of inpatient may still be appropriate:

- Leaving against medical advice (AMA)
- Unexpected death
- Transfer of the patient
- Unexpected clinical improvement
- Election of hospice care

- **When care is expected to span less than two midnights** but Acute, Intermediate, or Critical criteria are met, it may be appropriate to initially assign a status of observation.

NOTE: CMS has defined the following exceptions as being appropriate for inpatient status regardless of length of stay:

- Mechanical Ventilation, established during current hospitalization
- Surgical procedure or intervention found on the CMS Inpatient Only List

- **When it is undecided as to whether care will span two midnights**, the reviewer may use the level of care met as a guide to determine whether the patient is appropriate as an inpatient or observation status. The criteria consider multiple factors, including clinical severity, comorbidity, and risk, when differentiating between the Observation and Acute levels of care. These factors impact the length of stay. A patient meeting criteria at the Acute levels of care can generally be expected to have a length of stay of greater than two midnights and thus could be assigned to inpatient status. Patients who meet criteria at the Observation level of care are more appropriate for observation status.

(Centers for Medicare & Medicaid Services, Title 42; Part 412; Subpart A - Admissions. 2023; Centers for Medicare & Medicaid Services. Title 42; Part 422; Subpart C-Benefits and Beneficiary Protections. 2023; Centers for Medicare & Medicaid Services, In: BFCC-QIO 2 Midnight Claim Review Guideline. 2022)

2:

Initial Review criteria are a level of care determination tool, intended to be used as **real-time** decision support in the emergency department to identify if observation or inpatient hospital level services are warranted. They help the reviewer determine whether a patient is appropriate for observation or inpatient admission at the time a decision to admit the patient is being made. Initial Review criteria evaluate **only data that are available at the time the decision is being made**. This may include previously provided interventions or the results of laboratory, imaging, and other tests. Initial Review may be appropriate when bridge or holding orders (e.g., admit to telemetry) are in place. These orders are intended to address the patient's needs until full treatment and medication orders are written.

Initial Review criteria **should not be used** retrospectively or once the decision to admit has been made (e.g., when full treatment and medication orders have been written). If the reviewer has sufficient information to complete an Episode Day 1 review, an Initial Review need not be completed.

Note: While Initial Review enables identification of the level of care, it is not intended to, and should not be used as a substitute for an Episode Day 1 review, which will generally include specific, evidence-based interventions and

intensity of service requirements.

For further clarification, see the Acute Level of Care Review Process.

3:

Expected progress after day 2

- N-Acetylcysteine (NAC) in progress or completed
- Acetaminophen level decreasing
- Liver function tests (LFT) decreasing
- Mental status changes resolving

Consider:

- Discharge planning
- Referral to behavioral health clinician and appointment made within week (if not sooner)

Intentional overdose and:

- Close observation or 1:1 observation
- Psychiatry consult (if available)
- Behavioral Health consult

4:

Care Facilitation

- Intentional overdose and behavioral health services (**Inpatient BH program**)
- Prolonged oxygen therapy expected: arrange home oxygen therapy (**Home, Home Care**)
- Prolonged ventilator weaning expected: arrange transfer to vent weaning program (**SAC, LTAC**)

Social Determinants of Health (SDOH)

Social Determinants of Health (SDOH) are conditions in a person's environment that may affect their health. Social needs related to SDOH may impact the decision to admit, care coordination during hospital stay, as well as discharge destination. Children and caregivers should be screened for SDOH-related barriers to health to mitigate any unmet social and psychosocial factors impacting the health of the child and family unit (Kreuter et al., Annu Rev Public Health 2021, 42: 329-44; Sokol et al., Pediatrics 2019, 144:). The following social needs should be assessed on admission to determine which interventions are relevant to the individual child and their family support system. This list may not be all inclusive and all relevant SDOH factors should be considered. Any social need determined to be relevant to a given child and family should be planned for throughout the hospital stay.

*Interventions should be considered for all patients and families if social need is relevant

Alcohol or substance use

- Coordinate care with outpatient primary care or behavioral health providers*
- Refer to addiction social support systems*
- Provide information about community resources and programs for family-centered treatment (e.g., intensive case management, detox, outpatient, residential treatment, day treatment, and medication assisted treatment)
- Provide information about home visitation programs (e.g., Parent-Child Assistance Program (PCAP) for pregnant and parenting individuals with substance use disorders aftercare) (Hildebrandt et al., Infant Ment Health J 2020, 41: 677-96; Vanderplasschen et al., Front Psychiatry 2019, 10: 186)
- Provide information about Cognitive Behavioral Therapy (CBT) Multidimensional Family Therapy (MDFT) and/or Pharmacotherapy for youth with co-occurring emotional/mental disorders (Substance Abuse and Mental Health Administration, Treatment Considerations for Youth and Young Adults with Serious Emotional Disturbances and Serious Mental Illnesses and Co-occurring Substance Use. 2021)

Child maltreatment, abuse, or intimate partner abuse - sexual and/or psychological abuse

- Arrange for alternative housing situation, foster care, or shelter if home environment is unsafe*
- Assist with creating a long-term safety plan or make appropriate referral*
- Refer to child protective services (CPS) and other domestic violence services if warranted*
- Refer for psychosocial support if warranted*
- Provide community resources and legal information
- Provide information on parent training programs such as the Nurse Family Partnership Program (NFP) (Nurse-Family Partnership National Resources for Families. 2021; Gubbels et al., Int J Environ Res Public Health 2019, 16:)

End of life planning, arrange palliative care or hospice

- Consider palliative care for children with end stage disease
- Consider hospice care for terminally ill children with a life expectancy of less than 6 months

Environmental exposure to toxic substances and other physical hazards

Lead poisoning:

- Provide community resources for primary prevention and secondary prevention strategies (e.g., blood lead level testing and follow up (Centers for Disease Control and Prevention, Lead Poisoning Prevention. 2019)

Tobacco use (e.g., cigarettes, E-cigarettes, vaping):

- Provide education and/or brief counseling about tobacco use cessation and the risk of second-hand smoke (U.S. Department of Health and Human Services, Tobacco Use in Children and Adolescents: Primary Care Interventions. 2021)

Financial resources strained or limited

Food insecurity:

- Provide information about local food banks and special nutritional programs (e.g., Special Supplemental Nutrition Program for Women, Infants, and Children (WIC), U.S. Department of Agriculture Summer Food Service Program (Testa and Jackson, Ann Epidemiol 2021, 58: 22-8; Berman et al., Pediatr Rev 2018, 39: 235-46)
- Refer to direct income assistance benefits and indirect income assistance programs (e.g., programs that improve access to basic living necessities such as provision of food, daycare, and fuel or rent supplements)

Income insecurity:

- Identify income resources (Pottie et al., CMAJ 2020, 192: E240-E54)
- Refer to direct income assistance benefits and indirect income assistance programs (e.g., programs that improve access to basic living necessities such as provision of food, daycare, and fuel or rent supplements)

Infants and children with Special Health Care Needs

- Provide anticipatory guidance and education to caregivers regarding medical condition and expected progress after discharge over the next few days, months, or years, as warranted (Dosman and Andrews, Paediatr Child Health 2012, 17: 75-80)*
- Refer for follow-up care to neonatologist or other medical subspecialist for infants or children with ongoing and/or chronic medical issues, (e.g., bronchopulmonary dysplasia or feeding dysfunction) (Pediatrics 2008, 122: 1119-26)*
- Refer for adaptive/assistive medical equipment and equipment for technology-dependent children, (e.g., home oxygen, ventilators, monitors, or tube feeding) (Pediatrics 2008, 122: 1119-26)*
- Refer for neurodevelopmental assessment for high-risk infants*
- Refer to post-discharge follow-up programs (e.g., Maternal, Infant, and Early Childhood Home Visiting Program, Early Head Start program for infants or toddlers under the age of 3, and pregnant individuals in need of medical, mental health, nutrition and education, or the Nurse-Family Partnership for mothers pregnant with their first child and for ongoing nurse care visits through the child's second birthday) (Nurse-Family Partnership National Resources for Families. 2021; Health Resources & Services Administration, Maternal and Child, The Maternal, Infant, and Early Childhood Home Visiting Program 2021)
- Refer for behavioral support and/or counseling for caregivers through therapy groups, telephone support, and/or peer support groups for caregivers of children with similar health needs (Reyes et al., Health Equity 2021, 5: 100-18; Substance Abuse and Mental Health Administration, Treatment Considerations for Youth and Young Adults with Serious Emotional Disturbances and Serious Mental Illnesses and Co-occurring Substance Use. 2021; Pierron et al., BMC Public Health 2018, 18: 1087)
- Refer to lactation consultant in the hospital and home visits for infants at risk for poor lactation or suboptimal infant breast-feeding behavior (Leeman et al., Pediatr Qual Saf 2019, 4: e130)
- Refer for respite services, if warranted

Mental health co-morbidities involved

- Arrange appointment with primary care provider or behavioral health specialist upon discharge*
- Coordinate care with outpatient behavioral health providers*
- Provide information about suicide prevention, including the National Suicide Prevention hotline*
- Refer for psychiatric services for suicide risk screening and intervention
- Refer to school-based mental health interventions, if applicable (Fazel et al., Lancet Psychiatry 2014, 1: 377-87)
- Provide information about Intensive Community-based treatment (e.g., Functional Family Therapy, Intensive Child and Adolescent and Psychiatric Services, Multidimensional Family Therapy, and Intensive Case Management)
- Transfer to day treatment program, residential addiction services or partial hospitalization when warranted

Risk for treatment nonadherence

- Provide information about educational and behavioral support initiatives for children at risk of medication nonadherence and their caregivers, such as text message reminders to take medication and electronic medication dispensers
- Provide information about peer support groups for children and/or families with poor insight into illness or noncompliance with treatment (Mental Health America, Peer Support: Research and Reports 2021; National Institute for Health and Care Excellence (NICE). Psychosis and schizophrenia in adults: prevention and management. CG178. London: NICE; 2014.)
- Refer to interventions to prevent medical relapse, or readmission, and to enhance medication compliance

Risk for readmission

- Consider transfer to an alternative level of care such as subacute care
- Consider home care services
- Consider early childhood home visiting programs (e.g., Early Head Start; Early On; Nurse Family Partnership; and the Maternal, Infant, and Early Childhood Home Visiting Program (Health Resources & Services Administration, Maternal and Child, The Maternal, Infant, and Early Childhood Home Visiting Program 2021; Nurse-Family Partnership National Resources for Families. 2021)
- Consider telehealth and web-based training programs (e.g., telepsychiatry or telebehavioral health and mobile health interventions (J Am Acad Child Adolesc Psychiatry 2017, 56: 875-93)
- Consider Intensive in-home behavioral health services
- Provide patient and family with condition-specific information including appropriate planning for prevention of RSV infection and appropriate immunizations

Special Health Risk Behaviors in adolescence and young adulthood

- Arrange follow-up appointment with primary care provider and/or specialists upon discharge*
- Coordinate care with outpatient behavioral health providers*
- Provide information on mentoring programs, (e.g., Big Brothers Big Sisters of America)
- Refer for Intensive Community-Based treatment for children with comorbid behavioral health issues
- Refer to community resources for teen pregnancy prevention/family planning (use of birth control, subsequent pregnancies) (Maness et al., J Adolesc Health 2016, 58: 636-43)

For eating disorders:

- Provide information on nutrition services including education and resources for diet management, physical activity, and promotion of healthy eating attitudes and behaviors (Gato-Moreno et al., Int J Environ Res Public Health 2021, 18;; Al-Khudairy et al., Cochrane Database Syst Rev 2017, 6: Cd012691)
- Provide information on outpatient psychosocial interventions and Family-Based Treatment (Lock and La Via, J Am Acad Child Adolesc Psychiatry 2015, 54: 412-25)

For parenting resources:

- Provide information on Family Resource Centers and Parenting support groups for young, first-time parents, (e.g., National Parent Helpline) (National Parent and Youth Helpline)
- Provide information on community resources to improve parenting skills (face-to-face or by telephone) (U.S. Department of Health and Human Services, Parenting Resources. 2021)

Unstable living environment or homelessness expected upon discharge

- Assist with health insurance application

- Provide information about homeless shelter application to caregivers if applicable
- Refer to local housing coordinator or case manager for immediate link to permanent supportive housing and coordinated access system (Pottie et al., CMAJ 2020, 192: E240-E54)
- Refer to federally sponsored outpatient programs
- Refer to Intensive Case Management Intervention, Assertive Community Treatment, or other Intensive Community Treatment (Ponka et al., PLoS One 2020, 15: e0230896; Vijverberg et al., BMC Psychiatry 2017, 17: 284)

5:

Hemodynamically stable patients who require at least 6 hours, and for certain conditions, up to 48 hours, of treatment or assessment pending a decision regarding the need for additional care. Observation level of care services may be provided in designated observation units or on a hospital floor.

Note: InterQual® Level of Care criteria are intended to be used to support a patient-specific clinical determination of medical necessity and not for final billing or reimbursement decisions. Final payment decisions consider a variety of factors, such as clinical judgment, medical necessity, payer policies and contracts, and state and national regulations.

6:

Standards of care when administering N-Acetylcysteine (NAC) include ongoing monitoring of acetaminophen levels and liver function tests.

7:

The Rumack-Matthew Nomogram is a commonly used tool to assess the risk of hepatotoxicity and determine whether the administration of N-acetylcysteine (NAC) is appropriate in cases of a single, acute acetaminophen (APAP) ingestion. Most patients who receive NAC, the antidote for APAP overdose, within 8 hours of ingestion will benefit from its hepatotoxicity protective effects and will not develop acute liver failure. NAC should be administered immediately once an APAP overdose is suspected, as early administration may prevent liver damage. Many patients will present more than 8 hours after APAP ingestion - delays in care of 24 hours or more after an acute ingestion are common - and NAC administration for those late-presenting patients also may be beneficial in preventing liver necrosis. NAC can be given either orally or intravenously. Formal NAC protocols are available, but the dosage, route of administration, and duration of treatment, may vary based on patient status, laboratory values, and center-specific guidelines (Darr and Sussman, Clin Liver Dis 2020, 24: 121-9; Bateman and Dear, Toxicol Res (Camb) 2019, 8: 489-98; Fisher and Curry, Adv Pharmacol 2019, 85: 263-72; McNeil Consumer Healthcare, Guidelines for the Management of Acetaminophen Overdose. 2015; Rumack et al., Arch Intern Med 1981, 141: 380-5).

8:

Acetaminophen (APAP) levels typically peak about 4 hours after ingestion regardless of the formulation. The Rumack-Matthew nomogram is a useful tool for determining when to administer N-acetylcysteine (NAC) following a single, acute ingestion, and is based on both the APAP level and hours post ingestion (Fisher and Curry, Adv Pharmacol 2019, 85: 263-72; McNeil Consumer Healthcare, Guidelines for the Management of Acetaminophen Overdose. 2015; Rumack et al., Arch Intern Med 1981, 141: 380-5).

9:

Hemodynamically stable patients who require treatment, assessment, or intervention every 4 to 8 hours.

Note: InterQual® Level of Care criteria are intended to be used to support a patient-specific clinical determination of medical necessity and not for final billing or reimbursement decisions. Final payment decisions are based on a variety of factors including medical necessity, payer rules, and contracts.

10:

There are a variety of clinical features associated with acetaminophen toxicity including right upper quadrant pain, nausea, and vomiting (Hendrickson and McKeown, Rosen's emergency medicine: concepts and clinical practice, Ninth edition. ed. 2018).

11:

Hepatotoxicity from acetaminophen overdose is acute and characterized by marked elevation of serum alanine

aminotransferase (ALT) or aspartate aminotransferase (AST) (often greater than 1000 U/L(16.7 μ kat/L), which starts increasing within 24 to 36 hours, and peaks around 72 hours after overdose. The degree of aminotransferase elevation is almost synonymous with the degree of hepatocellular damage. For patients who receive N-acetylcysteine (NAC) within the first 8 hours after an acute overdose, the risk of hepatotoxicity is less than 5%(0.5), whereas delays beyond 10 hours are associated with an increased risk of hepatic injury (Bhushan and Apte, Am J Pathol 2019, 189: 719-29; Bunchorntavakul and Reddy, Clin Liver Dis 2018; 22(2): 325-346).

12:

Hemodynamically unstable patients (or those with the potential to become unstable) who require treatment, assessment, or intervention every 1 to 2 hours.

Note: InterQual® Level of Care criteria are intended to be used to support a patient-specific clinical determination of medical necessity and not for final billing or reimbursement decisions. Final payment decisions are based on a variety of factors including medical necessity, payer rules, and contracts.

13:

In case of delayed presentation following acetaminophen (APAP) overdose, findings of hepatotoxicity may already be present. The King's College Hospital (KCH) screening tool is the most widely used tool to define the risk of acute liver failure. It was modified several years after its release to increase sensitivity. The KCH tool includes laboratory and clinical findings associated with advanced hepatotoxicity and helps identify those patients that may require liver transplantation. It should be noted that advanced liver toxicity may result in a decrease in AST and ALT rather than continued elevation (McPhail et al., Clin Gastroenterol Hepatol 2016; 14(4): 516-525 e515; e543-e545; Bernal et al., Lancet 2002, 359: 558-63; O'Grady et al., Gastroenterology 1989, 97: 439-45).

14:

Hepatic encephalopathy (HE) is a complication of acute or chronic liver disease that is primarily characterized by changes in personality, consciousness, cognition, and motor function. West Haven criteria for grading HE includes (Montrief et al., Am J Emerg Med 2019, 37: 329-37; Weissenborn, Drugs 2019, 79: 5-9; Ferenci, Gastroenterol Rep (Oxf) 2017, 5: 138-47):

- Grade I - Lack of awareness, anxiety, short attention span, euphoria, inability to add/subtract
- Grade II - Disorientation for time, inappropriate behavior, lethargy/apathy, obvious personality change, dyspraxia, or asterixis
- Grade III - Confusion, bizarre behavior, gross disorientation, responsive to verbal stimuli, somnolence to semi-stupor
- Grade IV - Coma

15:

Examples of Grade III or IV hepatic encephalopathy may include:

- Grade III - Confusion, bizarre behavior, gross disorientation, responsive to stimuli, somnolence to semi-stupor
- Grade IV - Coma

16:

Acute mental status change may include coma, stupor, obtundation, confusion, disorientation, delirium, or increasing lethargy (Shenvi et al., Ann Emerg Med 2020, 75: 136-45).

17:

The CMS Two-Midnight Rule generally requires that the medical record support a patient's need for hospital level services **AND** that the patient has a hospital stay of at least two midnights.

For patients who are in observation status and do not meet Observation responder criteria on Episode Day 2, apply Episode Day 2 criteria at the Observation, Acute, Intermediate or Critical level of care in the appropriate subset to demonstrate the continued need for hospital-based services.

NOTE: For review of a surgical patient, Post-Operative Day 1 equates to Episode Day 2.

(Centers for Medicare & Medicaid Services, Title 42; Part 412; Subpart A - Admissions. 2023; Centers for Medicare & Medicaid Services. Title 42; Part 422; Subpart C-Benefits and Beneficiary Protections. 2023; Centers for Medicare & Medicaid Services, In: BFCC-QIO 2 Midnight Claim Review Guideline. 2022)

18:

Selection of this criteria point indicates that the patient is responding to treatment and is clinically stable for transfer or discharge. To determine the most appropriate post-acute level of care, see discharge criteria.

19:

Practice patterns differ regarding the expected duration of N-acetylcysteine (NAC). While the exact time of completion may vary based on the chosen protocol, it is generally considered safe to discontinue treatment once acetaminophen levels normalize or become undetectable, and liver function tests, INR, and the patient's clinical condition return to normal (Darr and Sussman, Clin Liver Dis 2020, 24: 121-9; Bateman and Dear, Toxicol Res (Camb) 2019, 8: 489-98; McNeil Consumer Healthcare, Guidelines for the Management of Acetaminophen Overdose. 2015).

20:

When a criteria point states "within acceptable limits," it refers to either the patient's normal baseline, a newly established baseline, or parameters that the medical practitioner determines are acceptable.

21:

Oral intake should be at least equivalent to the patient's maintenance fluid requirements or within parameters which the medical practitioner deems appropriate. Maintenance fluid requirements should be increased in the presence of dehydration or insensible loss. Maintenance fluid rates are based on ideal body weight and reflect the minimum amount of fluid required to maintain hydration. Rates may be decreased in older patients or those with a history of renal impairment or heart failure (Scales, Br J Nurs 2014, 23).

22:

Hemodynamically stable patients who require treatment, assessment, or intervention every 2 to 4 hours.

Note: InterQual® Level of Care criteria are intended to be used to support a patient-specific clinical determination of medical necessity and not for final billing or reimbursement decisions. Final payment decisions are based on a variety of factors including medical necessity, payer rules, and contracts.

23:

Noninvasive positive pressure ventilation (NIPPV), also known as NIV, provides respiratory support by application of a tightly fitting facial mask, nasal mask, or helmet rather than an endotracheal tube or tracheostomy. In some cases, the use of NIPPV can avoid the need for endotracheal intubation and decreases the risk of barotrauma, lung injury, and/or infection. NIPPV is commonly delivered by a bilevel positive airway pressure ventilator (BiPAP), a continuous positive airway pressure device (CPAP), or a mechanical ventilator. Supplemental oxygen can be delivered at concentrations approximating 100%(1.0).

The decision to provide respiratory support via NIPPV, and the modality to provide NIPPV, is based upon the patient's specific clinical findings (e.g., medical condition leading to respiratory failure, underlying comorbidities, clinical progression, improvement). Examples of NIPPV devices are volumetric (i.e., deliver a defined volume), barometric (i.e., deliver a defined pressure), and combined (i.e., deliver defined volumes and pressure). The terminology for NIPPV delivery systems may differ between equipment manufacturers and provider organizations. InterQual® criteria do not differentiate between the different NIPPV modalities.

Whether or not high-flow nasal cannula (HFNC) should be classified as a component of NIPPV is unclear, and there is conflicting evidence in the medical literature. Where HFNC is an appropriate modality, InterQual® defines this in a specific criteria point. As such, NIPPV does not include HFNC (Hackett, A., PulmCCM 2018; Nardi et al., F1000Res 2017, 6: 290; Osadnik et al., Cochrane Database Syst Rev 2017, 7: CD004104; Allison and Winters, Emerg Med Clin North Am 2016, 34: 51-62; Gregoretti et al., Crit Care Clin 2015, 31: 435-57).

24:

Oxygen therapy is the administration of oxygen at concentrations greater than ambient air (room air: 21%(0.21)) with the intent of treating and/or preventing the symptoms and manifestations of hypoxia. The oxygen concentration or percentage (FiO₂) delivered varies with the manufacturer's design, oxygen flow rate, the patient's respiratory rate, and tidal volume. Actual inspired FiO₂ with nasal cannula varies significantly with the patient's minute ventilation and pattern of breathing. For patients at risk for CO₂ retention, where a precise inspired FiO₂ is required, the Venturi mask is the preferred method. In general, patients who are very ill or have respiratory

disease may require considerably higher flow rates to achieve the desired FiO_2 . The following are estimates of O_2 delivered at the associated flow rates:

LOW-FLOW SYSTEMS

Nasal cannula

Room air 21%(0.21)

1L 24%(0.24) 4L 36%(0.36)

2L 28%(0.28) 5L 40%(0.40)

3L 32%(0.32) 6L 44%(0.44)

Simple oxygen masks

- 35-50%(0.35-0.50) FiO_2 (5-10 L/min)
- Flow rates are usually maintained at 5 L/min or more to avoid accumulation of CO_2 in the mask

Venturi masks

- 24-50%(0.24-0.50) FiO_2 (4-12 L/min)

Partial rebreathing masks

- 40-70%(0.40-0.70) FiO_2 (6-10 L/min)

Non-rebreathing masks

- 60-80%(0.60-0.80) FiO_2 (minimum flow of 10 L/min)

HIGH-FLOW SYSTEMS

Air-entrainment masks/nebulizers

- 24-40%(0.24-0.40) FiO_2

Heated, humidified high-flow concentrating nasal cannula (HFNC)

- 24-100%(0.24-1.0) FiO_2

25:

Patients presenting with Acetaminophen (APAP) overdose may not show signs of immediate toxicity and liver function test (LFT) abnormalities are usually not detectable sooner than 24 hours post ingestion. Aspartate aminotransferase (AST) or alanine aminotransferase (ALT) levels in excess of 1000 U/L (16.7 $\mu\text{kat/L}$) are indicative of severe hepatotoxicity. When the time of ingestion is known, the Rumack-Matthew nomogram is used to estimate the likelihood of hepatotoxicity in patients greater than 12 years of age. Treatment of APAP overdose with N-acetylcysteine should be initiated within 8 hours of acetaminophen ingestion. (Fisher and Curry, Adv Pharmacol 2019, 85: 263-72).

The clinical presentation of acetaminophen toxicity is divided into 4 stages (Bhushan and Apte, Am J Pathol 2019, 189: 719-29):

- Stage I - 12 to 24 hours post ingestion; symptoms include nausea, vomiting, diaphoresis, and anorexia.
- Stage II - 24 to 72 hours post ingestion; symptoms have decreased but AST, ALT, bilirubin, and prothrombin time become abnormal.
- Stage III - 72 to 96 hours post ingestion; peak abnormalities are seen and AST levels as high as 30,000 U/L (501 $\mu\text{kat/L}$) are possible. Less than 1%(0.01) of patients in stage III will develop fulminant hepatotoxicity.
- Stage IV - 4 days to 2 weeks post ingestion; hepatic abnormalities returning to near normal by days 7-8.

26:

Respiratory interventions include ventilator/noninvasive positive pressure ventilation (NIPPV) setting changes, chest physiotherapy, suctioning, and nebulizer or metered-dose inhaler (MDI) treatments.

27:

The CMS Two-Midnight Rule generally requires that the medical record support a patient's need for hospital level services **AND** that the patient has a hospital stay of at least two midnights.

For patients who are in observation status beyond the second midnight (Episode Day 3) and who do not meet Observation responder criteria, apply Episode Day 3 Inpatient criteria in the appropriate condition-specific or general subset.

Note: Patients who meet criteria **at any of** the Inpatient levels of care **based on medically necessary hospital-based services and supported by clinical documentation would be appropriate for inpatient status;** those who

do not meet criteria would be appropriate for secondary review.

(Centers for Medicare & Medicaid Services, Title 42; Part 412; Subpart A - Admissions. 2023; Centers for Medicare & Medicaid Services. Title 42; Part 422; Subpart C-Benefits and Beneficiary Protections. 2023; Centers for Medicare & Medicaid Services, In: BFCC-QIO 2 Midnight Claim Review Guideline. 2022)

28:

The discharge screen is a resource tool and not criteria. Referring to the discharge screen at the initiation of discharge planning is recommended.

29:

The home environment and safety assessment will normally include an evaluation of the patient's pre- and post-hospitalization functional level, physical layout of the home (e.g., entrances and exits, stairs, access to the community), identification of unsafe conditions (e.g., scatter rugs, missing handrails, oxygen use and smoking, lack of fire safety devices, inadequate lighting, heating, and cooling), factors that may trigger symptoms (e.g., secondhand smoke, poor food choices, dysfunctional family dynamics), sanitation hazards (e.g., lack of electricity, running water, refrigeration, inadequate toilet facilities, presence of insects or rodents), and the need for adaptive equipment (e.g., walker, handrails for the tub or shower, elevated toilet seat).

30:

Ensuring the patient and/or caregiver understands all aspects of the condition and can assume responsibility for self-care is crucial in preventing readmission. Assessing a patient and/or caregiver's level of understanding after education has been provided can be difficult. The teach-back method is an effective way of providing education at the appropriate level and can also be used to assess the learner's comprehension. To use the teach-back method, discuss key points in common terms and avoid using medical jargon or unfamiliar terms. After the education has been delivered, ask the learner to repeat what was learned. Gaps or misinterpretations in the learner's explanation will pinpoint areas where communication may have failed and provide opportunity for clarification.

31:

Medication reconciliation is a formal process or technique used by health care providers and pharmacists to identify the most complete and accurate list of all medications a patient is taking at times of transitions in care (e.g., upon hospital admission, transfer from one unit to another during hospitalization, or discharge from the hospital to home or another facility). The goals of this process are to ensure medication and dosages are appropriate for the patient, resolve discrepancies in drug regimens, and ultimately prevent medication errors and reduce adverse drug events. Medication reconciliation is a Joint Commission National Patient Safety goal. Coordinating information when a patient is transferred to another setting, service, practitioner, or level of care ensures accurate medications are listed. The process is comprised of the following (Hospital: National Patient Safety Goals. 2020):

- Obtain an external list of medications (e.g., medications taken prior to admission)
- Develop a list of current medications and add them to the medical record
- Compare the medications from the external list to the current list
- Clarify inconsistencies/discrepancies (e.g., omissions, duplications, contraindications, unclear information, and changes)
- Develop a list of medications to be prescribed at discharge or transfer
- Communicate the new list to the patient and/or appropriate caregiver(s)
- Ensure that patient and/or caregiver(s) understand the medication information upon discharge

Specific medication issues identified as being problematic include missing medication information from transfer orders, lack of information on medications provided in the acute setting, incomplete medication records, discrepancies between hospital regimen and discharge summary, and missing information pertaining to the patient's tolerance of a medication regimen. Although medication reconciliation is an important aspect in patient safety, there is a lack of consensus and evidence about the best effective methods of implementing this process. Physician-led and electronic medication reconciliation in hospitals are effective strategies to reduce medication discrepancies, however, the impact of these interventions is uncertain due to the low quality of evidence (Choi and Kim, J Clin Pharm Ther 2019, 44: 932-45; Redmond et al., Cochrane Database Syst Rev 2018, 8: CD010791). Trained pharmacy technicians, under the direction of a licensed pharmacist, may be an option for developing and expanding medication reconciliation processes (Irwin et al., Hosp Pharm 2017, 52: 44-53).

32:

Close follow-up of older adults, pediatric, or at-risk patients after discharge can help minimize hospital readmission and total health care costs (Rising et al., Ann Emerg Med 2013, 62: 145-50; Frei-Jones et al., Pediatr Blood Cancer 2009; 52(4): 481-485; Kripalani et al., JAMA 2007; 297(8): 831-841). Studies have shown that for greater than 50% (0.50) of patients re-hospitalized within 30 days of discharge, there was no visit to a physician's office between the time of discharge and re-hospitalization (Hernandez et al., JAMA 2010, 303: 1716-22; Jencks et al., N Engl J Med 2009; 360(14): 1418-1428). Efforts to reduce readmissions should focus on patients known to be at high risk and patients who have frequent visits to the emergency department (ED). Patients at high risk may have multiple ED visits prior to readmission (Rising et al., Ann Emerg Med 2013, 62: 145-50).

33:

Outpatient management contraindicated refers to the following:

- The patient whose medical condition is so fragile that going to the clinic or medical practitioner's office for treatment would put the patient at risk
- The patient with extensive equipment (e.g., ventilators, specialized beds) where travel outside the home would be cumbersome and places the patient at risk
- The treatment goal is to adapt the home environment for increased independence (e.g., an individual who is blind who needs mobility education)

34:

Outpatient management unavailable refers to the following:

- The patient whose treatment (e.g., dressing changes) is so extensive that it takes too long or is too frequent (e.g., twice daily) to be done in an outpatient clinic or medical practitioner's office
- The scheduling of the treatment falls outside the medical practitioner's office or clinic hours
- Travel distance to receive care is excessive (e.g., greater than 1 hour)
- The service is unavailable in the outpatient setting

35:

Skilled nursing services refer to services that must be provided by a licensed nurse qualified to assess and monitor patient condition(s) and provide medical treatment and/or teaching to patients who have skilled nursing needs. Examples of Home Care level of care interventions include skilled assessment, intervention, or education for any of the following:

- New treatment or medication regimen
- Adjustment in the treatment or medication regimen
- Infusion therapy administration, teaching, or access device management
- Management and evaluation of a care plan for patients with an active comorbidity and multiple care needs
- Chronic disease requiring a disease management program (e.g., asthma, heart failure, chronic obstructive pulmonary disease, diabetes)
- End-stage disease, hospice, or palliative care

36:

Skilled nursing interventions refer to services that are provided or supervised by a qualified or licensed professional when the care needs of the patient require assessment, observation, monitoring, and/or education to achieve the medically desired result. Close observation and assessment by nursing may be necessary at this level of care. This includes nursing care such as medication administration (excludes routine), vital signs monitoring, and/or other interventions not listed.

37:

The Home Care criteria are used for a patient who has been admitted or is expected to be admitted for home care. When criteria are met for this level of care and it is not available in your area, we recommend you use the next higher level of care.

Home care can include specialty services provided by a psychiatric nurse following a specialized treatment plan developed by a psychiatrist or advanced practice registered nurse. It is intended for treatment and monitoring of a patient's psychiatric condition that prevents the patient from leaving the house or makes leaving the house

extremely difficult or unsafe (Centers for Medicare & Medicaid Services. Chapter 7, In: Medicare benefit policy manual. 2022). Home care can also be used for a patient with severe and persistent mental illness with a history of nonadherence to medication.

Evaluation and Treatment

Programming may differ based upon legislative and geographical variances and is subject to organizational policy; however, at a minimum, it should include:

- Care coordination with treating physician and other providers
- Clinical assessment every visit
- Education every visit
- Medication assessment, teaching, and management every visit
- Medication reconciliation initiated within first visit
- Psychosocial assessment within first visit
- Substance evaluation within first 2 visits

For those patients who are covered under a Medicare benefit, the Affordable Care Act mandates that a certifying physician or allowed practitioner have a face-to-face visit with the patient within 90 days prior to the start of care, or within 30 days after the start of care. There must be documentation of the patient's condition that supports the need for home care services and the requirement of homebound status (Centers for Medicare & Medicaid Services. Chapter 7, In: Medicare benefit policy manual. 2022).

38:

Support system includes friends, family, social services, health care delivery providers or agencies, sponsors, clergy, and self-help groups.

39:

The initial clinical assessment is performed by a licensed professional and includes a comprehensive review of the patient's presenting diagnosis and a review of body systems. Identification of current and potential medical needs and health problems can be identified by the clinical assessment. The clinical assessment includes:

- History and physical exam (e.g., vital signs, height, weight)
- Pain assessment includes cause, intensity, quality, onset, duration, and effects on quality of life (e.g., activities of daily living (ADLs), instrumental activities of daily living (IADLs), and interpersonal relationships). Pain relievers should also be included
- Nutritional and hydration status
- Functional ability
- Safety and infection control measures
- Prescribed and over-the-counter (OTC) medications, herbal supplements, and home remedies
- Patient's and/or caregiver's understanding of medication use, dosing, side effects, and adherence to the medication or treatment regimen
- Patient's and/or caregiver's understanding of the illness or disease process and potential long-term complications
- Patient's and/or caregiver's coping strategies to deal with the illness or injury and ability to follow the plan of care

40:

For specific level of care recommendations for the treatment of psychiatric and substance use disorders, refer to the InterQual® Adult and Geriatric Psychiatry, InterQual® Child and Adolescent Psychiatry, or InterQual® Substance Use Disorders products.

41:

Goal-directed therapy includes one or more of the following:

- Activities of daily living (ADLs) (e.g., feeding, dressing, bathing, grooming, toileting, functional mobility)
- Bed mobility or transfers (e.g., transfers to chair, toilet, commode, tub, shower, car)
- Cognitive, speech, language, or swallowing retraining
- Home and lifestyle modification (e.g., assessment and training focused on the home, job, school, or work environment)
- Pain management techniques (e.g., assessment and interventions used to relieve pain or decrease its intensity)

- Pulmonary rehabilitation (e.g., energy conservation, speech, breathing re-education, postural drainage, percussion, or vibration)
- Positioning and splinting
- Range of motion and strengthening
- Wheelchair mobility
- Ambulation

42:

Rehabilitation potential refers to the probability that therapy and medical goals are realistic and attainable based on patient's prior level of function, severity of illness or injury, and the extent of impairments.

43:

Functional assistance levels are based upon the patient's function during tasks and activities necessary to return to household mobility or ambulation. The functional assistance level required for each individual task or activity (e.g., mobility, activities of daily living (ADLs)) may vary.

The following terms are commonly used in the post-acute setting:

- Independent - Patient can safely and within a reasonable amount of time perform a task (or developmentally appropriate task) without physical or cognitive assistance or supervision
- Modified Independent - Patient performs an activity with a supportive device, adaptive equipment, and/or prosthetic or orthotic device. Additional time may be required to complete the activity and/or there are safety (risk) considerations
- Supervision - Patient performs an activity with standby or distant supervision or setup. Verbal cueing or coaxing, without physical contact or setup of items and application of orthoses may be required when patient's safety awareness is impaired
- Minimum or limited Assistance - Patient performs at least 75%(0.75) of an activity and requires some physical contact to steady, guide, or move
- Moderate or extensive Assistance - Patient performs at least 50%(0.50) of an activity and requires physical assistance for functional mobility or ADLs
- Maximum Assistance - Patient performs 25%(0.25) to 50%(0.50) of an activity and requires physical assistance for functional mobility or ADLs
- Total Assistance or dependence - Patient performs less than 25%(0.25) of an activity and may require total assistance for functional mobility or ADLs

44:

The intensity of rehabilitation therapies is a driver for admission to an acute inpatient rehabilitation facility. Documentation should reflect the need for required and ongoing intensive therapies. The determination of whether a patient requires and can tolerate an intensive rehabilitation program of at least 15 hours is not based on any one single piece of documentation. All clinical documentation should be evaluated by the reviewer to determine whether the patient is demonstrating the need for services and has the ability to tolerate them. Patients who do not require this level of intensive therapy should be treated at a lower level of care. Rehabilitation therapies should be reasonable and medically appropriate and should not exceed a patient's level of tolerance. If there are clinical features which preclude the patient's ability to tolerate the intensity threshold of at least 15 hours per week, secondary review would be appropriate (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021; Miller and Forer, Pm r 2021, 13: 535-9; Forrest et al., Medicine (Baltimore) 2019, 98: e17096).

45:

Comorbid conditions include new onset or worsening behavioral symptoms, heart failure, deep vein thrombosis, uncontrolled diabetes, immunocompromised states, malignant or end-stage disease, malnutrition, renal insufficiency or end-stage renal disease, infection, ventilator dependence, noninvasive positive pressure ventilation or respiratory insufficiency, complex wound care, and new functional impairment(s) with limitation.

46:

Ventilation includes invasive ventilation and noninvasive positive pressure ventilation (NIPPV), including continuous positive airway pressure (CPAP) and bilevel positive airway pressure (BiPAP).

47:

Notable causes for prolonged mechanical ventilation include:

- Ventilator-acquired pneumonia
- Renal failure
- Heart failure
- Exacerbation of chronic obstructive pulmonary disease (COPD)
- Chest wall disorder
- Neuromuscular diseases
- Traumatic brain injury
- Sepsis
- Complications following cardiac surgical procedures

Major contributing factors for failure to wean include (Ambrosino and Vitacca, Multidiscip Respir Med 2018, 13: 6; Davies et al., Br J Anaesth 2017, 118: 563-9):

- Increased work of breathing
- Impaired respiratory drive
- Inspiratory muscle weakness

48:

Long-Term Acute Care (LTAC) is a recognized level of care designation by the Centers for Medicare and Medicaid Services (CMS) for acute care hospitals. It is defined by federal statutes as a level of care for patients requiring an average length of stay of 25 days or more (Centers for and Medicaid Services, Fed Regist 2018, 83: 41144-784). Patients who require a short stay for treatment may be more appropriate for the subacute or skilled nursing facility (SNF) level of care.